NCE EXAM PREP

Practice Exam 03

Domain 3

Treatment Planning & Continuity of Care

26 Practice Questions | 34 minutes

A, B, C answer choices

Answer Key and Explanations included

> **Instructions.** Select the single best answer for each question. Record your answers separately. This domain practice set is drawn from Full-Length Practice Exam 01. Use the suggested 34-minute practice timer. Do not consult Part B until you complete Part A. These domain timings are editorial practice allocations, not official NBCC section limits.

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PART A — QUESTIONS

Choose one answer (A, B, or C) per question.

Question 1

Which statement is written as a measurable treatment objective rather than a broad goal or counselor intervention?

A) The client will use two coping strategies during at least four work shifts each week for the next month

B) The client will improve emotional regulation and feel more confident at work

C) The counselor will teach diaphragmatic breathing and cognitive reframing during sessions

Question 2

> A client with posttraumatic symptoms requests immediate, detailed trauma processing. During the past week, the client has had frequent dissociative episodes, cannot reliably use grounding skills, and lacks a safe place to stay at night.

How should these facts affect the initial treatment plan?

A) Begin intensive trauma processing because the client's request establishes readiness

B) Emphasize stabilization and immediate supports before reconsidering intensive trauma processing

C) Exclude trauma-related goals permanently because dissociation makes trauma treatment ineffective

Question 3

> An adult client meets criteria for generalized anxiety disorder. When the counselor names the diagnosis, the client looks uneasy and says that in the client's family, a mental health label can imply personal weakness.

What is the best approach to reviewing the diagnosis for treatment planning?

A) Avoid discussing the diagnosis further and record that the client rejected it

B) Explain the evidence and purpose in accessible language, invite the client's perspective, and discuss cultural meaning

C) Emphasize that diagnostic terminology is objective and should not be influenced by family beliefs

Question 4

A client treated for obsessive-compulsive symptoms has met the behavioral targets for ten weeks and independently manages occasional symptom increases. Both counselor and client had agreed that these indicators would prompt a termination review. The client worries that ending weekly sessions means support will disappear.

Which response best supports an appropriate transition?

A) Continue weekly counseling for an open-ended period to prevent the client's discomfort

B) Terminate at the end of the current session because the criteria have already been met

C) Review the gains, plan a gradual ending if useful, and clarify booster and reentry options

Question 5

A 10-year-old frequently leaves the assigned area during independent schoolwork. The child, caregiver, and school counselor agree that the behavior is most likely when tasks are long. The child enjoys earning drawing time.

Which behavioral management plan is best designed for consistent use across settings?

A) Remove drawing materials whenever the child leaves the area, with each adult deciding how long they remain unavailable

B) Tell the child to show better self-control and review progress whenever another incident occurs

C) Define a brief work interval, reinforce remaining in the area with drawing time, and record the behavior consistently

Question 6

A client entered counseling for panic symptoms that interfered with commuting. At the eighth-week review, the client is commuting independently, uses coping skills without prompting, and has met the agreed symptom and functioning targets. The client is unsure whether counseling should end.

What is the most appropriate way to determine readiness for termination?

A) Continue weekly sessions until the client reports no anxiety in any situation

B) End services immediately because the original behavioral target has been met

C) Review the agreed criteria and remaining needs with the client, then decide on a taper or termination plan together

Question 7

Which treatment-plan entry is an intervention?

A) The client will reduce conflict-related outbursts from four per week to one per week within eight weeks

B) The client will strengthen the ability to manage anger in close relationships

C) The counselor will use role-play to help the client practice assertive requests during weekly sessions

Question 8

A college client in counseling for anxiety reports escalating food restriction, dizziness, and rapid weight loss. The counselor recommends prompt evaluation by an eating-disorder specialty team. The client agrees to coordination and signs a release that permits sharing information relevant to the referral.

What should the counselor do next to support continuity of care?

A) Send the specialty team the complete counseling record because a release has been signed

B) Make a timely referral and share the pertinent clinical information authorized by the client

C) Pause all counseling contact until the specialty team completes its evaluation

Question 9

A 15-year-old is stepping down from an intensive outpatient program after improved mood and school attendance. Continued success depends on medication follow-up, school accommodations, family support, and weekly outpatient counseling. The adolescent and guardian consent to coordination.

Which transition approach best supports continuity?

A) Give the family a list of providers and let each service develop unrelated goals

B) Arrange a warm handoff with shared transition goals, confirmed appointments, and defined provider roles

C) Delay discharge until the adolescent can function without school or family supports

Question 10

Which combination provides the most useful basis for evaluating treatment effectiveness at planned intervals?

A) Repeated symptom or behavior data, functioning indicators, and the client's view of progress

B) The counselor's impressions and the number of sessions the client has attended

C) The original diagnosis and whether the client completes every assigned activity

Question 11

A client with depression set an objective of walking outdoors for 20 minutes on five days each week. After three weeks, the client reports that knee pain limits walking but says seated exercise is manageable and still supports the broader goal of increasing activity.

What is the best treatment-planning response?

A) Keep the walking objective because changing it would make the original outcome data unusable

B) Revise the activity objective collaboratively to remain measurable and attainable while addressing the same broader goal

C) Replace the activity goal with a pain-focused diagnosis before making any further treatment changes

Question 12

A client has sustained improvement in depression and is preparing to discharge from outpatient counseling. The client recalls that withdrawal from friends and missing morning classes preceded the last worsening episode.

Which discharge-plan element would most directly support maintenance of gains?

A) A promise to return to counseling only if symptoms become as severe as before

B) A written plan linking early warning signs to coping steps, support contacts, and follow-up options

C) A summary listing every intervention the counselor used during the course of treatment

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Question 13

A client completing outpatient treatment for alcohol use disorder identifies payday gatherings with former drinking partners as a high-risk situation. The client has a supportive cousin and has practiced leaving risky settings early.

Which relapse-prevention plan is most complete?

A) Identify warning signs, plan alternatives for payday, rehearse contacting the cousin, and specify what to do after a lapse

B) Advise the client to avoid all social events indefinitely and regard any lapse as treatment failure

C) Ask the client to rely on motivation and return only if regular alcohol use resumes

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Question 14

A client has made little progress on a goal requiring three evening activities each week. The client recently became the primary caregiver for an older relative and says the original schedule is no longer realistic, although the overall goal remains important.

Which response best reflects appropriate plan revision?

A) Collaboratively adjust the objective and supports to fit the new caregiving demands while preserving the valued goal

B) Retain the objective for another month so that progress can be compared with the original baseline

C) Remove the goal because a change in circumstances indicates that the client is not ready for treatment

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Question 15

> Assessment findings support social anxiety disorder for a client whose main goal is to participate in staff meetings. The client currently avoids speaking and rates anticipated distress as high but manageable with support.

Which initial plan best integrates the assessment findings with the client's goal?

A) Develop a graded sequence of meeting-related speaking tasks and track distress and participation

B) Focus on general self-esteem until the client reports feeling fully confident in all social settings

C) Recommend an unstructured social group without connecting attendance to the workplace goal

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Question 16

> A counselor in independent practice is creating a professional will in case sudden incapacity prevents communication with clients.

Which provision most directly promotes continuity of care?

A) Designating a qualified professional to secure records and notify clients about care options

B) Authorizing a family member to review records and decide which clients need further treatment

C) Directing that all clinical records be destroyed immediately if the counselor becomes incapacitated

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Question 17

> A couple requests conjoint counseling after one partner disclosed ongoing intimidation, monitoring of communications, and fear of retaliation for speaking openly. The fearful partner says it would not be safe to discuss the relationship honestly in joint sessions.

How should the counselor use this information in planning services?

A) Begin conjoint counseling with a rule that neither partner may discuss sessions afterward

B) Use communication-skills training in the first joint session to reduce the power imbalance

C) Avoid conjoint work at this time and develop a safety-focused referral or individual service plan

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Question 18

> An autistic adult seeks help managing workplace overload. The client explains that fluorescent lighting and unpredictable verbal instructions contribute to shutdowns and prefers written agendas and concrete planning. The client wants to remain in the current job.

Which treatment-planning response is most appropriate?

A) Prioritize increasing tolerance of sensory discomfort before discussing workplace changes

B) Build concrete regulation and self-advocacy objectives around the client's sensory needs and communication preferences

C) Recommend leaving the job because environmental triggers cannot be addressed through counseling

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Question 19

A client managing chronic pain says that spiritual practices are an important source of endurance but has not said whether these practices should be part of counseling. The counselor wants the plan to reflect the client's worldview without making assumptions.

What is the best planning step?

A) Add participation in a faith community as an objective because spirituality is a stated strength

B) Ask how, if at all, the client wants spiritual resources incorporated into goals and supports

C) Keep spirituality out of the plan because it is outside the scope of professional counseling

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Question 20

At intake, a client describes insomnia, conflict with a sibling, overdue bills, and worry about job performance. The client says, “If I could sleep, I could think clearly enough to handle the rest,” and reports feeling ready to change the bedtime routine.

Which planning response best supports collaborative prioritization?

A) Begin with the overdue bills because financial stress is the most concrete concern

B) Agree to target sleep first while noting how the other concerns will be monitored

C) Address the sibling conflict first because interpersonal stress may be causing the insomnia

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Question 21

At a scheduled six-week review, a client's standardized anxiety scores and work attendance are unchanged. The client has attended consistently but says the current plan focuses on relaxation while the main concern is avoiding difficult conversations with a supervisor.

What is the best response to the review findings?

A) Continue the current plan unchanged because six weeks is too early to use outcome data

B) Discharge the client because the stable scores show that counseling is ineffective

C) Explore the mismatch with the client and revise the goals or approach while continuing to monitor outcomes

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Question 22

A client in stable outpatient treatment will relocate to another state in six weeks. The client wants continued counseling, has chosen a prospective provider, and authorizes limited coordination. The current counselor cannot practice in the destination state.

Which plan best supports the transition?

A) Coordinate the authorized handoff, confirm interim and emergency supports, and plan the ending before the move

B) Continue telehealth after the move until the client feels established, regardless of practice authority

C) End services now so the client has enough time to locate care without influence from the current counselor

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Question 23

A client referred for mild depressive symptoms says the most urgent concern is keeping housing after work hours were reduced. The counselor believes increasing pleasant activities could help mood, while the client wants short-term planning around housing stability first.

How should the counselor establish the initial focus?

A) Begin behavioral activation because the diagnosis should determine the first treatment target

B) Address only the housing concern and exclude depressive symptoms from the plan

C) Collaboratively prioritize housing stability while connecting mood support to the client's immediate needs and preferences

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Question 24

A counselor learns that surgery will require a six-week medical leave beginning next month. Several clients are in active treatment, and the agency has clinicians who can provide temporary coverage.

Which continuity plan is most appropriate?

A) Wait until the final session before leave to avoid creating unnecessary concern

B) Give all clients the covering clinicians' names and transfer every case without discussion

C) Discuss the interruption in advance, arrange individualized coverage or alternatives, and provide emergency contact procedures

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Question 25

What is the best foundation for an initial treatment plan?

A) A synthesis of assessment findings, diagnosis when applicable, client priorities, strengths, barriers, and context

B) A standard protocol selected solely from the diagnostic label before discussing the client's preferences

C) A list of every problem identified by the counselor, addressed simultaneously in order of clinical complexity

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Question 26

A Deaf client who uses American Sign Language begins counseling for adjustment-related stress. The client prefers a qualified interpreter and visual summaries of between-session tasks. The agency can provide both without changing the frequency of care.

How should the counselor incorporate this information into the treatment plan?

A) Replace the client's stated goals with communication goals before addressing the presenting concern

B) Use written communication only so that no third party is present during counseling

C) Document the communication accommodations and apply them while pursuing the client's chosen goals

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PART B — ANSWER KEY AND EXPLANATIONS

Answers match the numbered items in Part A.

Question 1 — Correct: A

Correct Answer: The client will use two coping strategies during at least four work shifts each week for the next month

Explanation: This statement specifies an observable client behavior, frequency, setting, and time frame. The broad improvement statement is a goal, while the statement describing what the counselor will teach is an intervention.

Question 2 — Correct: B

Correct Answer: Emphasize stabilization and immediate supports before reconsidering intensive trauma processing

Explanation: Frequent dissociation, limited grounding capacity, and unsafe housing indicate that intensive trauma processing may currently destabilize the client. The plan can prioritize safety and stabilization without permanently excluding later trauma-focused work.

Question 3 — Correct: B

Correct Answer: Explain the evidence and purpose in accessible language, invite the client's perspective, and discuss cultural meaning

Explanation: Collaborative diagnosis review includes transparent reasoning and attention to the client's understanding and cultural context. Treating the label as culturally neutral or ending the discussion would undermine informed participation in treatment planning.

Question 4 — Correct: C

Correct Answer: Review the gains, plan a gradual ending if useful, and clarify booster and reentry options

Explanation: The agreed indicators support a termination discussion, while the client's concern can be addressed through a planned transition. Reviewing gains and access options supports autonomy and continuity without making routine weekly care indefinite.

Question 5 — Correct: C

Correct Answer: Define a brief work interval, reinforce remaining in the area with drawing time, and record the behavior consistently

Explanation: This plan operationalizes the target behavior, adjusts the task demand, uses an identified reinforcer, and permits consistent monitoring. Vague expectations or inconsistent punishment make implementation and evaluation less reliable.

Question 6 — Correct: C

Correct Answer: Review the agreed criteria and remaining needs with the client, then decide on a taper or termination plan together

Explanation: The client has met the preidentified symptom and functioning targets, making a termination review appropriate. Readiness should be evaluated collaboratively, including any remaining needs and whether tapering would support the transition; neither zero anxiety nor abrupt termination is required.

Question 7 — Correct: C

Correct Answer: The counselor will use role-play to help the client practice assertive requests during weekly sessions

Explanation: An intervention states what the counselor will do to help the client reach an objective. The frequency-reduction statement is a measurable objective, and the broad ability statement is a treatment goal.

Question 8 — Correct: B

Correct Answer: Make a timely referral and share the pertinent clinical information authorized by the client

Explanation: The symptoms warrant prompt specialty evaluation, and the client's authorization permits focused coordination. A release does not make the entire record automatically necessary, and abruptly pausing all contact could create a gap before specialty care begins.

Question 9 — Correct: B

Correct Answer: Arrange a warm handoff with shared transition goals, confirmed appointments, and defined provider roles

Explanation: A coordinated step-down plan connects the adolescent to each needed service and clarifies responsibility across systems. A provider list alone leaves important gaps, while successful lower-level care may appropriately include ongoing school and family supports.

Question 10 — Correct: A

Correct Answer: Repeated symptom or behavior data, functioning indicators, and the client's view of progress

Explanation: Multiple sources gathered over time show both clinical change and its practical meaning to the client. Attendance alone, counselor impressions, or task completion cannot establish whether treatment is producing the intended outcomes.

Question 11 — Correct: B

Correct Answer: Revise the activity objective collaboratively to remain measurable and attainable while addressing the same broader goal

Explanation: New physical information affects the feasibility of the original behavior but not the client's broader aim of increasing activity. A measurable alternative preserves the treatment direction and permits continued evaluation without requiring a diagnostic change.

Question 12 — Correct: B

Correct Answer: A written plan linking early warning signs to coping steps, support contacts, and follow-up options

Explanation: The client has identified meaningful early indicators of worsening depression. Connecting those signs to specific actions and resources creates a practical maintenance and reentry plan rather than requiring severe deterioration before help is sought.

Question 13 — Correct: A

Correct Answer: Identify warning signs, plan alternatives for payday, rehearse contacting the cousin, and specify what to do after a lapse

Explanation: An effective relapse-prevention plan links a known trigger to alternatives, social support, practiced coping, and a response to a lapse. Treating a lapse as total failure can intensify recurrence, and motivation alone is not a sufficient continuity strategy.

Question 14 — Correct: A

Correct Answer: Collaboratively adjust the objective and supports to fit the new caregiving demands while preserving the valued goal

Explanation: A major contextual change has made the original schedule unrealistic, not the broader goal irrelevant. Revising the objective with the client keeps the plan attainable and responsive while maintaining direction.

Question 15 — Correct: A

Correct Answer: Develop a graded sequence of meeting-related speaking tasks and track distress and participation

Explanation: A graded, measurable plan directly addresses the avoidance maintaining social anxiety and the client's functional workplace goal. A diffuse focus on self-esteem or an unrelated unstructured activity is less closely tied to the assessment findings and desired outcome.

Question 16 — Correct: A

Correct Answer: Designating a qualified professional to secure records and notify clients about care options

Explanation: A professional will prepares for unexpected interruption by assigning appropriate responsibility for record security and client notification or referral options. Family access or automatic destruction could compromise confidentiality and prevent orderly continuity of care.

Question 17 — Correct: C

Correct Answer: Avoid conjoint work at this time and develop a safety-focused referral or individual service plan

Explanation: Ongoing intimidation and fear of retaliation make open participation in conjoint counseling unsafe and may increase risk. Communication training or session rules do not correct the coercive conditions described and should not substitute for safety-focused planning.

Question 18 — Correct: B

Correct Answer: Build concrete regulation and self-advocacy objectives around the client's sensory needs and communication preferences

Explanation: The plan should respond to the client's identified environmental triggers, preferred communication style, and goal of remaining employed. Requiring tolerance before considering accommodations or assuming job loss is necessary would disregard strengths and feasible contextual changes.

Question 19 — Correct: B

Correct Answer: Ask how, if at all, the client wants spiritual resources incorporated into goals and supports

Explanation: The counselor can recognize spirituality as a possible strength while allowing the client to define its relevance and form. Prescribing faith-community participation would make an unsupported assumption, whereas excluding spirituality would overlook a client-identified resource.

Question 20 — Correct: B

Correct Answer: Agree to target sleep first while noting how the other concerns will be monitored

Explanation: The client identifies sleep as the concern with the greatest immediate leverage and expresses readiness to work on it. Selecting that focus collaboratively while tracking the other problems respects client priorities without ignoring the broader case.

Question 21 — Correct: C

Correct Answer: Explore the mismatch with the client and revise the goals or approach while continuing to monitor outcomes

Explanation: Converging outcome and functioning data show little change, and the client identifies a plausible mismatch between the plan and the central problem. Collaborative revision uses the review findings constructively; unchanged scores do not by themselves require either automatic continuation or discharge.

Question 22 — Correct: A

Correct Answer: Coordinate the authorized handoff, confirm interim and emergency supports, and plan the ending before the move

Explanation: A planned handoff within the client's authorization reduces gaps while respecting the current counselor's practice limits. Continuing without authority is inappropriate, and immediate termination would unnecessarily remove support before relocation.

Question 23 — Correct: C

Correct Answer: Collaboratively prioritize housing stability while connecting mood support to the client's immediate needs and preferences

Explanation: Housing instability is the client's urgent concern and a major context for mood and functioning. A collaborative plan can address that priority while retaining relevant depression support, rather than allowing either diagnosis or a single social need to exclude the other.

Question 24 — Correct: C

Correct Answer: Discuss the interruption in advance, arrange individualized coverage or alternatives, and provide emergency contact procedures

Explanation: Advance notice and individualized planning reduce the risk of an avoidable gap in care. Automatically transferring every client would not account for client preference or clinical need, while waiting until the last session would limit safe transition options.

Question 25 — Correct: A

Correct Answer: A synthesis of assessment findings, diagnosis when applicable, client priorities, strengths, barriers, and context

Explanation: An initial plan should integrate clinical findings with the client's goals and the factors likely to support or obstruct progress. Diagnosis alone cannot determine an individualized plan, and attempting to address every problem at once weakens prioritization.

Question 26 — Correct: C

Correct Answer: Document the communication accommodations and apply them while pursuing the client's chosen goals

Explanation: A qualified interpreter and visual summaries make the plan accessible while preserving the client's autonomy and presenting priorities. Substituting communication goals or relying only on writing would disregard the client's stated needs and preferences.
